Referral packet F16: readable copy

Source: F16.pdf (fictional), 14 pages. Left side of each sheet is the original page, right side is a typed transcript. Black boxes are privacy redactions, not missing information.

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4	Patient face sheet	face_sheet	good	
5	History and Physical - page 1	h_and_p	good	
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8	Medication administration record (excerpt)	mar	good	
9	Respiratory therapy note	rt_note	good	
10	Physical therapy evaluation	pt_eval	good	
11	Nursing shift note	nursing_note	good	
12	Social work note	social_work	good	
13	Labs, imaging and vitals	labs	good	
14	Discharge medication list	med_list	good	

Fax cover sheet

Bayfront Regional Medical Center (fictional)
Case Management Department
FAX TRANSMITTAL
To: Illumination Health + Home - Recuperative Care Intake
Fax: 555-0199
From: Teresa Lindqvist, RN CM
Phone: 555-0161
Date: 10/06/2026 09:15
Pages (including cover): 14
Re: Recuperative care referral
Urgent [X] For Review [X] Please Reply [X]
Message
58M community-acquired pneumonia, improving. Room air, ambulating independently.
Hoping for d/c to recuperative care 10/06 PM. Please call CM with questions.
CONFIDENTIALITY NOTICE
This facsimile contains confidential health information. If received in error, notify sender and destroy.

-- FICTIONAL DEMO RECORD - not a real patient --

Invented for the Claude Impact Lab demo. Names, facilities and numbers are fictional.

Page 1: Fax cover sheet

fax_cover | orientation normal | legibility good

Bayfront Regional Medical Center (fictional)

Case Management Department

FAX TRANSMITTAL

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Fax: 555-0199
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Pages (including cover): 14
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Urgent **[X]** For Review **[X]** Please Reply **[X]**

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IHH Recuperative Care Referral Form

ILLUMINATION HEALTH + HOME

Recuperative Care Referral Form (RC0725-SH)

Referring Party

Hospital: Bayfront Regional Medical Center (fictional)
Department/Floor: 3 North Medical
Social Worker/Case Manager/Nurse: Teresa Lindqvist, RN CM
Phone: 555-0161
Email: tlindqvist@bayfront.example
Nursing Station: 555-0162
Authorized By: Dr. P. Okonkwo

Patient Information

Name: Daniel Doe
MRN: BR-6120873
Birth Date: 04/12/1968
Gender: M
Ethnicity: White
SSN (last 4): 0000
Insurance: Orange Coast Health Plan (fictional) Medi-Cal
Preferred Language: English
Housing Situation:
Currently Unhoused [X]
4+ episodes of homelessness in the past 3 years [ ]

Hospital Admission Details

Chief Complaint/Admitting Diagnosis: Community-acquired pneumonia, RLL
Admit Date: 10/02/2026
Any Wounds: Yes [ ] No [X]
Number/Location/Size/Stage:
Independent with Wound Care: Yes [ ] No [ ]
Home Health Ordered: Yes [ ] No [X]
PPD/TB Test or Chest X-Ray: Date: 10/02/2026 Result: CXR - RLL infiltrate, no cavitation

History of Health & Living Situation

O2: Yes [ ] No [X]
Self Administer All Meds: Yes [X] No [ ]
Continent of Bowel & Bladder: Yes [X] No [ ]
Colostomy/Ileostomy: Yes [ ] No [X]
Foley Catheter: Yes [ ] No [X]
Diabetes: Yes [ ] No [X]
Insulin: Yes [ ] No [X]
Diabetes Oral Meds: Yes [ ] No [X]
Active Infection: Yes [X] No [ ] pneumonia - On IV antibiotics: Yes
Ambulatory: Yes [X] No [ ]
Assistive Device: Yes [ ] No [X]
Hospice: Yes [ ] No [X]
Car: Yes [ ] No [X]
Service Animal: Yes [ ] No [X]

Behavioral and Mental Health Status

Invented for the Claude Impact Lab demo. Names, facilities and numbers are fictional.

Page 2: IHH Recuperative Care Referral Form

referral_form | orientation normal | legibility good

ILLUMINATION HEALTH + HOME

Recuperative Care Referral Form (RC0725-SH)

Referring Party

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Department/Floor: 3 North Medical
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Housing Situation:
Currently Unhoused **[X]**
4+ episodes of homelessness in the past 3 years [ ]

Hospital Admission Details

Chief Complaint/Admitting Diagnosis: Community-acquired pneumonia, RLL
Admit Date: 10/02/2026
Any Wounds: Yes [ ] No **[X]**
Number/Location/Size/Stage:
Independent with Wound Care: Yes [ ] No [ ]
Home Health Ordered: Yes [ ] No **[X]**
PPD/TB Test or Chest X-Ray: Date: 10/02/2026 Result: CXR - RLL infiltrate, no cavitation

History of Health & Living Situation

O2: Yes [ ] No **[X]**
Self Administer All Meds: Yes **[X]** No [ ]
Continent of Bowel & Bladder: Yes **[X]** No [ ]
Colostomy/Ileostomy: Yes [ ] No **[X]**
Foley Catheter: Yes [ ] No **[X]**
Diabetes: Yes [ ] No **[X]**
Insulin: Yes [ ] No **[X]**
Diabetes Oral Meds: Yes [ ] No **[X]**
Active Infection: Yes **[X]** No [ ] pneumonia - On IV antibiotics: Yes
Ambulatory: Yes **[X]** No [ ]
Assistive Device: Yes [ ] No **[X]**
Hospice: Yes [ ] No **[X]**
Car: Yes [ ] No **[X]**
Service Animal: Yes [ ] No **[X]**

Behavioral and Mental Health Status

Any limitations or challenges: No **[X]** Yes [ ]
Mental Health DX: Yes [ ] No **[X]** Type:
Non-compliant: Yes [ ] No **[X]**
Cognitive Impairment: Yes [ ] No **[X]**
Forgetful: Yes [ ] No **[X]**
Registered Sex Offender: Yes [ ] No **[X]**
Other: None

Length of Stay Authorized

Days: 14

Illumination Staff Only

Date Received:
Reviewed By:
Approved: Yes [ ] No [ ]
Date:
Authorization #:
Reason for Denial:
Admission Date/Time:
Exit Date:
Extension Request: Yes [ ] No [ ]

Referring Party Comments

On IV antibiotics: Yes (ceftriaxone/azithromycin).
Independent, walking halls. Needs a safe place to finish recovery.

-- FICTIONAL DEMO RECORD - not a real patient --

CalAIM Community Supports authorization request

CalAIM COMMUNITY SUPPORTS - AUTHORIZATION REQUEST
Service: Recuperative Care (Medical Respite)
Health Plan: Orange Coast Health Plan (fictional) - Medi-Cal
Member
Member Name: Daniel Doe
DOB: 04/12/1968
Member ID: OCH-55219036
Referring Provider
Facility: Bayfront Regional Medical Center (fictional)
Clinical
Primary Diagnosis: Pneumonia, unspecified organism (RLL)
ICD-10: J18.9
Member is experiencing homelessness [X]
Member at risk of hospitalization/re-hospitalization [X]
Requested Start Date: 10/06/2026
Requested Days: 14
Admit Date: 10/02/2026
Clinical summary: CAP improving, room air, independent ADLs.
Plan Use Only
Status: Pending review

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Invented for the Claude Impact Lab demo. Names, facilities and numbers are fictional.

Page 3: CalAIM Community Supports authorization request

calaim_auth | orientation normal | legibility good

CalAIM COMMUNITY SUPPORTS - AUTHORIZATION REQUEST

Service: Recuperative Care (Medical Respite)
Health Plan: Orange Coast Health Plan (fictional) - Medi-Cal

Member

Member Name: Daniel Doe
DOB: 04/12/1968
Member ID: OCH-55219036

Referring Provider

Facility: Bayfront Regional Medical Center (fictional)

Clinical

Primary Diagnosis: Pneumonia, unspecified organism (RLL)
ICD-10: J18.9
Member is experiencing homelessness **[X]**
Member at risk of hospitalization/re-hospitalization **[X]**
Requested Start Date: 10/06/2026
Requested Days: 14
Admit Date: 10/02/2026
Clinical summary: CAP improving, room air, independent ADLs.

Plan Use Only

Status: Pending review

-- FICTIONAL DEMO RECORD - not a real patient --

Patient face sheet

Bayfront Regional Medical Center (fictional) - PATIENT FACE SHEET
Patient: Daniel Doe
MRN: BR-6120873
Account #: 99120873
DOB: 04/12/1968
Sex: Male
Preferred Language: English
Ethnicity: Not Hispanic or Latino
Marital Status: Single
Address: NO FIXED ADDRESS (homeless)
Mailing: 1234 Example Ave, Santa Ana, CA 92700 (shelter mail drop)
Phone: 555-0100 (message)
Encounter
Admit Date/Time: 10/02/2026 13:05 (ED arrival 10/02/2026 08:40)
Admit Type: Inpatient
Unit/Bed: 3N-318
Attending: Patience Okonkwo, MD
Insurance
Primary: Orange Coast Health Plan (fictional) - Medi-Cal
Subscriber: Self
Emergency Contact
None listed

-- FICTIONAL DEMO RECORD - not a real patient --

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Page 4: Patient face sheet

face_sheet | orientation normal | legibility good

Bayfront Regional Medical Center (fictional) - PATIENT FACE SHEET

Patient: Daniel Doe
MRN: BR-6120873
Account #: 99120873
DOB: 04/12/1968
Sex: Male
Preferred Language: English
Ethnicity: Not Hispanic or Latino
Marital Status: Single
Address: NO FIXED ADDRESS (homeless)
Mailing: 1234 Example Ave, Santa Ana, CA 92700 (shelter mail drop)
Phone: 555-0100 (message)

Encounter

Admit Date/Time: 10/02/2026 13:05 (ED arrival 10/02/2026 08:40)
Admit Type: Inpatient
Unit/Bed: 3N-318
Attending: Patience Okonkwo, MD

Insurance

Primary: Orange Coast Health Plan (fictional) - Medi-Cal
Subscriber: Self

Emergency Contact

None listed

-- FICTIONAL DEMO RECORD - not a real patient --

History and Physical - page 1

HISTORY AND PHYSICAL
Date: 10/02/2026
CC
Cough, fever, shortness of breath x 4 days
HPI
58 y/o M, unhoused x 2 yrs (sleeps in a tent near Example Creek trail), presents with productive cough, fevers and dyspnea on exertion x 4 days. In ED T 102.1F, HR 112, SpO2 89% RA improving to 94% on 2L NC. CXR: RLL consolidation.
Prior level of function: independent, no assistive device, walks daily to meal program.
PMH
- Tobacco use (30 pack-years)
- No known COPD (no prior PFTs)
Meds PTA
None
Allergies
NKDA
Social
Tobacco 1/2 ppd. EtOH rarely, last drink >1 month ago. Denies drug use.

-- FICTIONAL DEMO RECORD - not a real patient --

Invented for the Claude Impact Lab demo. Names, facilities and numbers are fictional.

Page 5: History and Physical - page 1

h_and_p | orientation normal | legibility good

HISTORY AND PHYSICAL

Date: 10/02/2026

CC

Cough, fever, shortness of breath x 4 days

HPI

58 y/o M, unhoused x 2 yrs (sleeps in a tent near Example Creek trail), presents with productive cough, fevers and dyspnea on exertion x 4 days. In ED T 102.1F, HR 112, SpO2 89% RA improving to 94% on 2L NC. CXR: RLL consolidation. Prior level of function: independent, no assistive device, walks daily to meal program.

PMH

- Tobacco use (30 pack-years)
- No known COPD (no prior PFTs)

Meds PTA

None

Allergies

NKDA

Social

Tobacco 1/2 ppd. EtOH rarely, last drink >1 month ago. Denies drug use.

-- FICTIONAL DEMO RECORD - not a real patient --

History and Physical - page 2

Exam

T 101.4F HR 108 BP 128/76 RR 22 SpO2 94% 2L NC
Crackles RLL. No accessory muscle use.
Alert, AOx4, calm and cooperative.

Assessment/Plan

1. Community-acquired pneumonia, RLL (CURB-65 = 1) - ceftriaxone 1 g IV q24h + azithromycin 500 mg IV daily; blood and sputum cultures; droplet precautions pending respiratory viral panel
2. Acute hypoxemia - O2 to keep SpO2 >= 92%, wean as tolerated
3. Tobacco use - nicotine patch, cessation counseling
4. Dispo - SW/CM consult (unhoused)

Signed: Patience Okonkwo, MD 10/02/2026 15:10

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Page 6: History and Physical - page 2

h_and_p | orientation normal | legibility good

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Signed: Patience Okonkwo, MD 10/02/2026 15:10

-- FICTIONAL DEMO RECORD - not a real patient --

Hospitalist progress note

HOSPITALIST PROGRESS NOTE - HD#5
Date: 10/06/2026 07:40
S: Cough much improved, less sputum. No dyspnea walking in hall. Eating well.
O: T 98.4F HR 82 BP 122/74 RR 16 SpO2 95% RA. Few RLL crackles. AOx4.
Afebrile since 10/03 18:00. WBC 14.8 -> 8.9.
A/P:
1. CAP RLL - clinically improved. IV ceftriaxone/azithromycin STOPPED 10/04; transitioned to PO amoxicillin-clavulanate 875 mg BID through 10/09. Azithromycin course completed 10/05. No IV antibiotics needed. PIV removed 10/05.
2. Hypoxemia - resolved. On room air since 10/04, ambulatory SpO2 95% RA. No home O2 needed.
3. Isolation - respiratory viral panel negative 10/02; droplet precautions discontinued 10/02. No isolation.
4. Dispo - medically stable for d/c to recuperative care today. PCP f/u and repeat CXR in 6 wks.
Signed: Patience Okonkwo, MD

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Page 7: Hospitalist progress note

progress_note | orientation normal | legibility good

HOSPITALIST PROGRESS NOTE - HD#5

Date: 10/06/2026 07:40
S: Cough much improved, less sputum. No dyspnea walking in hall. Eating well.
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2. Hypoxemia - resolved. On room air since 10/04, ambulatory SpO2 95% RA. No home O2 needed.
3. Isolation - respiratory viral panel negative 10/02; droplet precautions discontinued 10/02. No isolation.
4. Dispo - medically stable for d/c to recuperative care today. PCP f/u and repeat CXR in 6 wks.
Signed: Patience Okonkwo, MD

-- FICTIONAL DEMO RECORD - not a real patient --

Medication administration record (excerpt)

MEDICATION ADMINISTRATION RECORD - ANTIBIOTICS
Patient: Daniel Doe MRN: BR-6120873
Ceftriaxone 1 g IV q24h | Start 10/02 11:30 | Given 10/02 11:30, 10/03 11:00, 10/04 10:45 | DISCONTINUED 10/04 12:00
Azithromycin 500 mg IV daily | Start 10/02 12:00 | Given 10/02, 10/03, 10/04 | DISCONTINUED 10/04 12:00
Azithromycin 500 mg PO daily | 10/05 09:00 given | Course complete 10/05
Amoxicillin-clavulanate 875 mg PO BID | Start 10/04 21:00 | Given 10/04 21:00, 10/05 09:00, 10/05 21:00, 10/06 09:00 | through 10/09
IV access
Peripheral IV R forearm - removed 10/05 14:00, site clean
Notes
Pt self-administered PO doses with RN observation 10/05 and 10/06; states dose and schedule correctly.

-- FICTIONAL DEMO RECORD - not a real patient --

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Page 8: Medication administration record (excerpt)

mar | orientation normal | legibility good

MEDICATION ADMINISTRATION RECORD - ANTIBIOTICS

Patient: Daniel Doe MRN: BR-6120873
Ceftriaxone 1 g IV q24h | Start 10/02 11:30 | Given 10/02 11:30, 10/03 11:00, 10/04 10:45 | DISCONTINUED 10/04 12:00
Azithromycin 500 mg IV daily | Start 10/02 12:00 | Given 10/02, 10/03, 10/04 | DISCONTINUED 10/04 12:00
Azithromycin 500 mg PO daily | 10/05 09:00 given | Course complete 10/05
Amoxicillin-clavulanate 875 mg PO BID | Start 10/04 21:00 | Given 10/04 21:00, 10/05 09:00, 10/05 21:00, 10/06 09:00 | through 10/09

IV access

Peripheral IV R forearm - removed 10/05 14:00, site clean

Notes

Pt self-administered PO doses with RN observation 10/05 and 10/06; states dose and schedule correctly.

-- FICTIONAL DEMO RECORD - not a real patient --

Respiratory therapy note

RESPIRATORY THERAPY NOTE
Date: 10/05/2026 16:00
O2 weaned to room air 10/04 08:00. Ambulatory oximetry 10/05: rest SpO2 96% RA, after walking 200 ft SpO2 95% RA, no desaturation below 94%.
No nebulizer or inhaler treatments required. Incentive spirometer 1500 mL, pt independent with use.
No home O2 indicated.
Signed: G. Mbatha, RRT

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Page 9: Respiratory therapy note

rt_note | orientation normal | legibility good

RESPIRATORY THERAPY NOTE

Date: 10/05/2026 16:00
O2 weaned to room air 10/04 08:00. Ambulatory oximetry 10/05: rest SpO2 96% RA, after walking 200 ft SpO2 95% RA, no desaturation below 94%.
No nebulizer or inhaler treatments required. Incentive spirometer 1500 mL, pt independent with use.
No home O2 indicated.
Signed: G. Mbatha, RRT

-- FICTIONAL DEMO RECORD - not a real patient --

Physical therapy evaluation

PHYSICAL THERAPY EVALUATION
Date: 10/05/2026 11:00
Bed mobility: independent
Sit<>stand: independent
Transfers: independent, toilet and shower chair transfer independent
Gait: 200 ft x 2, independent, no assistive device, steady; 1 standing rest break
Stairs: 12 steps with one rail, independent
Balance: good static and dynamic
Assessment
At baseline. No skilled PT needs. No DME needed.
Recommendation
D/c to recuperative care or community without PT follow-up.
Signed: H. Varga, PT, DPT

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Page 10: Physical therapy evaluation

pt_eval | orientation normal | legibility good

PHYSICAL THERAPY EVALUATION

Date: 10/05/2026 11:00
Bed mobility: independent
Sit<>stand: independent
Transfers: independent, toilet and shower chair transfer independent
Gait: 200 ft x 2, independent, no assistive device, steady; 1 standing rest break
Stairs: 12 steps with one rail, independent
Balance: good static and dynamic

Assessment

At baseline. No skilled PT needs. No DME needed.

Recommendation

D/c to recuperative care or community without PT follow-up.
Signed: H. Varga, PT, DPT

-- FICTIONAL DEMO RECORD - not a real patient --

Nursing shift note

NURSING SHIFT NOTE
Date: 10/06/2026 06:30
Pt AOx4. Independent with all ADLs. Showered independently 10/05 PM, gathered own supplies. Continent of bowel and bladder, toilets independently.
Self-administers PO meds, teach-back done on amoxicillin-clavulanate schedule.
Calm and cooperative. No behavioral concerns this admission.
Isolation status: none (standard precautions).
Room air, SpO2 95%. Afebrile.
Signed: C. Duarte, RN

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Page 11: Nursing shift note

nursing_note | orientation normal | legibility good

NURSING SHIFT NOTE

Date: 10/06/2026 06:30
Pt AOx4. Independent with all ADLs. Showered independently 10/05 PM, gathered own supplies. Continent of bowel and bladder, toilets independently.
Self-administers PO meds, teach-back done on amoxicillin-clavulanate schedule.
Calm and cooperative. No behavioral concerns this admission.
Isolation status: none (standard precautions).
Room air, SpO2 95%. Afebrile.
Signed: C. Duarte, RN

-- FICTIONAL DEMO RECORD - not a real patient --

Social work note

SOCIAL WORK NOTE
Date: 10/05/2026 13:30
Pt engaged and pleasant, AOx4. Denies SI/HI. Denies auditory or visual hallucinations.
No psychiatric history.
No conservatorship. Not enrolled in FSP. Not a registered sex offender per pt.
Unhoused x 2 yrs; tent and belongings at encampment. Agreeable to recuperative care,
then housing navigation referral.
Signed: Teresa Lindqvist, RN CM / S. Achterberg, MSW

-- FICTIONAL DEMO RECORD - not a real patient --

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Page 12: Social work note

social_work | orientation normal | legibility good

SOCIAL WORK NOTE

Date: 10/05/2026 13:30
Pt engaged and pleasant, AOx4. Denies SI/HI. Denies auditory or visual hallucinations. No psychiatric history.
No conservatorship. Not enrolled in FSP. Not a registered sex offender per pt.
Unhoused x 2 yrs; tent and belongings at encampment. Agreeable to recuperative care, then housing navigation referral.
Signed: Teresa Lindqvist, RN CM / S. Achterberg, MSW

-- FICTIONAL DEMO RECORD - not a real patient --

Labs, imaging and vitals

LABS

10/02 | WBC 14.8 H | Hgb 13.9 | Plt 301 | Na 136 | K 4.0 | Cr 1.0 | BUN 18 | Procalcitonin 1.2 H
10/04 | WBC 11.2 H
10/06 | WBC 8.9 | Cr 0.9
10/02 Blood cultures x2: no growth at 72 hours (final pending)
10/02 Sputum culture: Strep pneumoniae, ceftriaxone and amoxicillin susceptible
10/02 Respiratory viral panel (incl. SARS-CoV-2 PCR): Negative
10/02 MRSA nares PCR: Negative
10/06 05:40 SARS-CoV-2 rapid antigen: Negative

IMAGING

10/02 CXR PA/lat: RLL consolidation, no effusion, no cavitation, no pneumothorax
10/05 CXR portable: improving RLL opacity

VITAL SIGNS - LAST 5 SETS

Date/Time	Temp (F)	HR	BP	RR	SpO2	O2
10/05 08:00	98.2	84	124/76	16	95%	RA
10/05 14:00	98.6	86	126/78	16	96%	RA
10/05 20:00	98.4	80	120/72	16	95%	RA
10/06 02:00	98.0	76	118/70	14	95%	RA
10/06 07:40	98.4	82	122/74	16	95%	RA

-- FICTIONAL DEMO RECORD - not a real patient --

Invented for the Claude Impact Lab demo. Names, facilities and numbers are fictional.

Page 13: Labs, imaging and vitals

labs | orientation normal | legibility good

LABS

10/02 | WBC 14.8 H | Hgb 13.9 | Plt 301 | Na 136 | K 4.0 | Cr 1.0 | BUN 18 | Procalcitonin 1.2 H
10/04 | WBC 11.2 H
10/06 | WBC 8.9 | Cr 0.9
10/02 Blood cultures x2: no growth at 72 hours (final pending)
10/02 Sputum culture: Strep pneumoniae, ceftriaxone and amoxicillin susceptible
10/02 Respiratory viral panel (incl. SARS-CoV-2 PCR): Negative
10/02 MRSA nares PCR: Negative
10/06 05:40 SARS-CoV-2 rapid antigen: Negative

IMAGING

10/02 CXR PA/lat: RLL consolidation, no effusion, no cavitation, no pneumothorax
10/05 CXR portable: improving RLL opacity

VITAL SIGNS - LAST 5 SETS

Date/Time | Temp (F) | HR | BP | RR | SpO2 | O2
10/05 08:00 | 98.2 | 84 | 124/76 | 16 | 95% | RA
10/05 14:00 | 98.6 | 86 | 126/78 | 16 | 96% | RA
10/05 20:00 | 98.4 | 80 | 120/72 | 16 | 95% | RA
10/06 02:00 | 98.0 | 76 | 118/70 | 14 | 95% | RA
10/06 07:40 | 98.4 | 82 | 122/74 | 16 | 95% | RA

-- FICTIONAL DEMO RECORD - not a real patient --

Discharge medication list

DISCHARGE MEDICATION LIST
Patient: Daniel Doe MRN: BR-6120873
- Amoxicillin-clavulanate 875 mg PO BID through 10/09/2026 (6 doses remaining)
- Nicotine patch 14 mg/24h daily
- Acetaminophen 650 mg PO q6h PRN fever/pain
- Guaifenesin 600 mg PO BID PRN cough
No IV medications. No home O2.
Patient able to self-administer all medications: Yes
Meds-to-beds: full supply dispensed by inpatient pharmacy at discharge

-- FICTIONAL DEMO RECORD - not a real patient --

Invented for the Claude Impact Lab demo. Names, facilities and numbers are fictional.

Page 14: Discharge medication list

med_list | orientation normal | legibility good

DISCHARGE MEDICATION LIST

Patient: Daniel Doe MRN: BR-6120873

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- Nicotine patch 14 mg/24h daily
- Acetaminophen 650 mg PO q6h PRN fever/pain
- Guaifenesin 600 mg PO BID PRN cough

No IV medications. No home O2.

Patient able to self-administer all medications: Yes

Meds-to-beds: full supply dispensed by inpatient pharmacy at discharge

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